

The Global Valproate Catastrophe: Pharmacological Toxicity, Corporate Accountability, and the Pursuit of Cross-Jurisdictional Legal Redress

The widespread and sustained prescription of sodium valproate—marketed globally under trade names including Epilim, Depakine, and Depakote—represents one of the most profound, multifaceted, and deeply entrenched iatrogenic disasters in modern medical and pharmacological history. Initially heralded as a revolutionary broad-spectrum anticonvulsant and mood stabilizer following its serendipitous discovery as an active pharmacological agent in France in the 1960s, the medication has since been inextricably and causally linked to devastating teratogenic outcomes. Most notably, maternal ingestion of sodium valproate during pregnancy is the primary catalyst for Fetal Valproate Spectrum Disorder (FVSD)—a condition sometimes colloquially or erroneously transcribed in lay discourse as "sodium evaporate excessive disorder"—which encompasses severe congenital malformations, profound neurodevelopmental delays, and life-altering cognitive impairments.

Despite peer-reviewed scientific literature and internal corporate pharmacovigilance data highlighting the physical risks as early as the 1980s, and the neurodevelopmental risks by the early 2000s, an intersection of corporate deflection, aggressive marketing, regulatory inertia, and fragmented state oversight allowed the continued exposure of pregnant women and women of childbearing potential (WOCBP) for several decades. The ongoing international fallout surrounding sodium valproate is vast, encompassing criminal indictments for manslaughter against the primary manufacturer, Sanofi, in France, collapsed group litigation in the United Kingdom, and highly contentious, delayed public inquiries in the Republic of Ireland.

This exhaustive research report provides a forensic analysis of the ongoing valproate crisis. It examines the contemporary pharmacological understanding of the drug's adverse effects—both for the primary user and the fetus—including emerging data on paternal exposure risks and male infertility. Furthermore, it scrutinizes the corporate conduct of Sanofi, detailing the extensive litigation, civil suits, and allegations of regulatory capture across France, the United Kingdom, and Ireland.

Particular analytical attention is devoted to the systemic, structural barriers to justice. In the Republic of Ireland, the State's strategic reliance on "non-statutory" investigative frameworks—a paradigm deliberately mirroring its historical management of institutional abuses perpetrated by the Roman Catholic Church—has severely complicated and protracted the pursuit of legal reparations for victims. Within this sociopolitical and legal context, the report explicitly explores the highly complex legal mechanisms available to individuals holding dual British and Irish citizenship who have been subjected to egregious medical negligence. This specifically addresses the clinical and legal nightmare of misdiagnosis, wherein a patient is erroneously placed on valproate for a condition they did not possess, and subsequently maintained on the drug off-label for intermittent migraines without adequate risk disclosure, resulting in catastrophic congenital defects in their offspring.

1. Pharmacological Profile and the Spectrum of Valproate Toxicity

Sodium valproate (IUPAC name: sodium 2-propylpentanoate) is a short-chain fatty acid derivative that exerts its therapeutic effects through a complex, multi-modal mechanism of action. It is primarily understood to potentiate the inhibitory neurotransmitter gamma-aminobutyric acid (GABA) by increasing its synthesis and decreasing its degradation, while simultaneously blocking voltage-gated sodium and calcium ion channels in the central nervous system. These mechanisms effectively raise the seizure threshold, making it a highly efficacious treatment for generalized and partial epilepsies, as well as a potent mood stabilizer for bipolar disorder. Furthermore, valproate functions as a potent histone deacetylase (HDAC) inhibitor. While this HDAC inhibition contributes to its neurological efficacy, it is also heavily implicated in its severe epigenetic and teratogenic profile, altering gene expression during crucial phases of embryonic development.

1.1 Current Adverse Effects and Long-Term Health Impacts

While designated by the World Health Organization (WHO) as an essential medicine for epilepsy and bipolar disorder, valproate carries a severe and systemic toxicity profile that extends far beyond its primary neurological indications. The ongoing international clinical consensus dictates that the drug should be utilized only when all other therapeutic alternatives have been exhausted or are clinically contraindicated.

Physiological System	Documented Adverse Clinical Effects	Diagnostic and Long-Term Implications
Hepatic System	Severe hepatotoxicity, elevated liver enzymes (SGOT, SGPT), liver injury	Can precipitate fulminant hepatic failure, which is occasionally fatal. The risk is significantly amplified in pediatric patients under two years of age or during aggressive initial dose ramping. Requires rigorous, ongoing liver function monitoring.
Gastrointestinal	Acute pancreatitis, persistent nausea, severe vomiting, abdominal pain	Valproate-induced pancreatitis can manifest with fatal hemorrhagic complications. General gastrointestinal distress is highly prevalent during the dose titration phase.
Neurological & Cognitive	Essential tremors, severe dizziness, persistent somnolence, encephalopathy	Exacerbates cognitive dulling and fatigue. Crucially, the abrupt withdrawal or cessation of valproate carries an acute risk of breakthrough seizures, status epilepticus, and Sudden Unexpected Death in Epilepsy

Physiological System	Documented Adverse Clinical Effects	Diagnostic and Long-Term Implications
		(SUDEP).
Endocrine & Metabolic	Substantial weight gain, Polycystic Ovary Syndrome (PCOS), alopecia, hyperandrogenism	Irregular or delayed menstruation is a frequent side effect. This requires immediate differential diagnosis to rule out unplanned pregnancy or valproate-induced PCOS, a condition permanently affecting endocrine health.
Hematological	Thrombocytopenia, unusual bruising, prolonged bleeding times	Requires routine hematological monitoring due to the distinct risk of impaired coagulation and blood disorders.
Musculoskeletal	Osteoporosis, osteopenia	Long-term, sustained treatment with sodium valproate is causally linked to reduced bone mineral density, significantly increasing the risk of spontaneous fractures in later life.

1.2 Teratogenicity: Fetal Valproate Spectrum Disorder (FVSD)

The most devastating international impact of sodium valproate is its catastrophic effect on the developing embryo and fetus. When ingested by a pregnant woman, the drug readily crosses the placental barrier, leading to a recognized clinical entity formally designated by the World Health Organization in 2019 as Fetal Valproate Spectrum Disorder (FVSD). This umbrella term, which encompasses the older diagnostic labels of Fetal Anticonvulsant Syndrome (FACS) and Fetal Valproate Syndrome (FVS), accounts for the entire continuum of physical and neurodevelopmental damages. In various public forums and layman testimonies, this condition is sometimes phonetically misunderstood or transcribed as "sodium evaporate excessive disorder," but the clinical reality of FVSD remains unequivocally recognized by the global medical establishment.

Epidemiological data, unchallenged by global regulatory agencies, indicates that infants exposed to valproate in utero face a 10% to 11% absolute risk of major congenital malformations. These physical deformities frequently include neural tube defects (such as spina bifida), facial dysmorphism (including cleft lip and cleft palate), severe congenital heart defects, limb abnormalities, and urogenital tract defects.

More insidiously, between 30% and 40% of children exposed to the drug in the womb will suffer from permanent, severe neurodevelopmental disorders. These cognitive impairments include Autism Spectrum Disorder (ASD)—with the risk of childhood autism increased approximately five-fold compared to the unexposed population—Attention Deficit Hyperactivity Disorder (ADHD), and significant reductions in baseline intelligence quotient (IQ). Studies consistently demonstrate that the IQ of school-aged children with a history of in utero valproate exposure is, on average, 7 to 10 points lower than that of children exposed to alternative antiepileptic drugs. The demographic scale of this pharmacological tragedy is staggering. In France alone, health

authorities and parliamentary estimates indicate that between 15,000 and 30,000 children have been born with valproate-induced disabilities, or were stillborn, since the drug's commercial introduction in 1967. In the United Kingdom, the Independent Medicines and Medical Devices Safety Review (the Cumberlege Review) estimated that up to 20,000 babies have been profoundly affected. In the Republic of Ireland, the Health Service Executive (HSE) estimates that up to 1,250 families have been catastrophically impacted, representing a massive per-capita exposure rate.

1.3 Emerging International Controversies: Paternal Exposure and Epigenetic Risks (2024–2026)

As global regulatory agencies progressively tightened restrictions on prescribing valproate to female patients of childbearing age, a new vector of intense international concern emerged regarding paternal exposure. A retrospective observational safety study, based on comprehensive electronic medical records from three Nordic countries and requested by the European Medicines Agency's (EMA) Pharmacovigilance Risk Assessment Committee (PRAC), indicated an elevated risk of neurodevelopmental disorders in children whose fathers were treated with valproate monotherapy in the three months prior to conception.

The data suggested a risk prevalence ranging from 5.6% to 6.3% for paternal valproate exposure, compared to a baseline risk of 2.5% to 3.6% for alternative antiepileptic drugs such as lamotrigine or levetiracetam. This prompted the UK's Medicines and Healthcare products Regulatory Agency (MHRA) to implement unprecedented, sweeping prescribing limitations in January 2024. These new regulations stipulated that no individual—male or female—under the age of 55 should be initiated on valproate without the independent endorsement of two distinct medical specialists confirming the absolute absence of viable therapeutic alternatives. (This protocol was slightly amended in 2025 to remove the two-expert requirement for *existing* male prescriptions, while retaining the stringent dual-signature requirement for all new male prescriptions and all female prescriptions). Additional research has pointed to potential male infertility and epigenetic changes, including testicular histone hyperacetylation altering sperm DNA methylation, thereby affecting subsequent generations.

Sanofi's corporate response to these emerging paternal risks was immediate and highly defensive. The manufacturer publicly challenged the Nordic safety study, asserting to health authorities and epilepsy charities that the data contained severe "limitations" and required extensive re-analysis before any definitive regulatory conclusions could be drawn. Sanofi stated that a "thorough data review process has shown that new analyses need to be performed". This institutional resistance mirrors the company's historical reluctance to proactively alter product labeling or admit fault without absolute, undeniable regulatory compulsion—a corporate posture that has fueled decades of ongoing corruption and negligence allegations globally.

2. The Off-Label Trap: Misdiagnosis, Migraine Prophylaxis, and the Breach of Duty

The legal, medical, and ethical complexities surrounding sodium valproate are profoundly compounded when the drug is prescribed off-label, or worse, secondary to a fundamental clinical misdiagnosis. While valproate is FDA-approved in the United States for the prevention of migraine headaches, its use for this indication has been drastically curtailed. Following the

publication of the NEAD (Neurodevelopmental Effects of Antiepileptic Drugs) study, which quantified the cognitive deficits in exposed children, the FDA decisively moved valproate to Pregnancy Category X for migraine prophylaxis. This absolute contraindication signifies that the catastrophic teratogenic risks to the fetus unequivocally outweigh any conceivable benefits for migraine treatment in pregnant women.

2.1 The Misdiagnosis Paradigm

A profound vector for medical negligence arises when a patient is erroneously diagnosed with a severe neurological or psychiatric condition—such as complex partial seizures, generalized epilepsy, or bipolar disorder—and is subsequently initiated on sodium valproate. If the initial diagnosis was fundamentally flawed (for instance, mistaking cardiac syncope, psychogenic non-epileptic seizures, or severe complex migraines for epileptic seizures), the very initiation of valproate represents a primary, egregious breach of the medical duty of care. The patient is subjected to the severe hepatic, metabolic, and neurological side effects of a toxic drug for a pathology they do not possess.

If the patient's true clinical presentation is eventually correctly identified as intermittent migraines, yet the attending physician maintains the valproate prescription off-label without executing a rigorous reassessment of the risk-benefit ratio, the liability deepens exponentially. In the United Kingdom and Ireland, the off-label use of valproate for migraines has occurred historically, though modern clinical guidelines strictly position it behind safer alternatives such as beta-blockers (propranolol), topiramate, or newer CGRP monoclonal antibodies.

2.2 The Failure of Informed Consent and the Post-Pregnancy Restriction Marketing

The core of the legal negligence in these scenarios lies in the failure to secure informed consent. Following the tightening of European regulations between 2014 and 2018, it became a strict regulatory requirement that women of childbearing potential prescribed valproate must be enrolled in a highly monitored Pregnancy Prevention Programme (PPP). This includes the mandatory use of highly effective contraception, an annual specialist review, and the completion of an Annual Risk Acknowledgement Form.

However, historical records and patient testimonies reveal a systemic failure to apply these safeguards to patients who were not explicitly pregnant or actively planning a pregnancy at the exact moment of prescription. Women misdiagnosed and prescribed valproate for intermittent migraines were frequently left off these critical warning registries. The assumption by medical professionals that a patient was not an immediate pregnancy risk led to the catastrophic omission of teratogenic warnings. Consequently, when these women did eventually conceive, they were completely oblivious to the fact that their daily migraine medication was inflicting severe structural and cognitive damage upon their unborn child.

Continuing a highly teratogenic antiepileptic drug for intermittent migraines in a dual UK/Irish female citizen of childbearing age, particularly without enrolling her in a mandatory PPP or documenting her explicit understanding of FVSD risks, constitutes a gross deviation from established pharmacological and legal protocols. Furthermore, the withdrawal of valproate in these misdiagnosed patients must be managed with extreme clinical delicacy. Abrupt cessation, even when the drug was prescribed erroneously, can trigger severe rebound headaches, medication-overuse headache (MOH) withdrawal symptoms, and broader neurological

destabilization, effectively trapping the patient in a cycle of chemical dependency induced by the initial medical negligence.

3. Corporate Accountability: Sanofi's Historical Conduct, Timelines, and Corruption Allegations

The global narrative of the sodium valproate tragedy is dominated by the relentless legal and public pursuit of its primary manufacturer, the French multinational pharmaceutical giant Sanofi (formerly Sanofi-Synthelabo and Sanofi-Aventis). The corporation has faced decades of accusations regarding aggressive marketing practices, the deliberate obfuscation of emerging scientific data, alleged regulatory capture, and the deployment of aggressive legal attrition tactics designed to exhaust plaintiffs and shield corporate assets.

3.1 The Chronology of Knowledge vs. Disclosure

The fundamental legal argument against Sanofi across all jurisdictions centers on the stark discrepancy between what the corporation knew regarding valproate's toxicity and what it actively disclosed to health authorities and patients.

Era	Scientific and Regulatory Developments	Sanofi's Stated Position / Corporate Action
1960s–1970s	Valproic acid's antiepileptic properties are discovered in France (1963). The drug is licensed in France (1967), the UK (1973 as Epilim), and Ireland (1975). Early animal studies show teratogenicity.	Sanofi successfully markets the drug globally as a breakthrough, highly effective broad-spectrum anticonvulsant.
1980s	Medical literature robustly identifies the physical teratogenic effects of valproate, specifically neural tube defects (spina bifida). French courts later confirm health officials knew of these physical risks by 1983.	Sanofi claims it proactively provided information about fetal malformation risks to health authorities in the 1980s and requested label modifications.
1990s	Geneticists and pediatricians increasingly recognize and codify the distinct facial and developmental markers of Fetal Valproate Syndrome (FVS).	Valproate continues to be heavily prescribed to women. The UK Committee on Safety of Medicines (CSM) opts not to heavily publicize risks to avoid "fruitless anxiety".
Early 2000s	Definitive clinical awareness emerges linking in utero valproate exposure to severe neurodevelopmental delays, learning disabilities, and autism	Sanofi asserts it informed health authorities of neurodevelopmental delay (NDD) risks in 2003, but claims the French Health Authority

Era	Scientific and Regulatory Developments	Sanofi's Stated Position / Corporate Action
	spectrum disorders (ASD).	(ANSM) refused to update patient leaflets until 2006.
2014–2018	The European Medicines Agency (EMA) enforces sweeping restrictions, declaring women are "still not always receiving the right information in a timely manner". The Pregnancy Prevention Programme (PPP) is mandated.	Sanofi complies with regulatory mandates but faces massive public backlash and the initiation of historic class-action lawsuits and criminal probes.

3.2 France: The Epicenter of Criminal Indictments and Class Actions France serves as ground zero for the Depakine scandal and the ensuing legal reckoning. In 2017, the French patient advocacy group APESAC initiated a landmark class-action lawsuit against Sanofi—the first of its kind in the French health sector. In a stunning escalation of corporate accountability, French prosecutors formally indicted Sanofi in August 2020 for involuntary manslaughter and aggravated fraud. The charges alleged that the company's sustained failure to adequately warn patients and physicians directly contributed to the deaths of infants and the severe, permanent disablement of tens of thousands more. At the same time, the French National Medicines and Health Products Agency (ANSM) was also indicted for "injuries and manslaughter by negligence," highlighting the systemic failure of state regulatory oversight.

In January 2022, a Paris court delivered a historic, precedent-setting ruling in the APESAC class action. The court found Sanofi definitively at fault for breaching its fundamental duty of care and failing to provide adequate information regarding congenital malformations (between 1984 and 2006) and neurodevelopmental disorders (between 2001 and 2006). Crucially, the court legally classified Depakine as a "defective product" during these specific timeframes. Concurrently, the French State, acknowledging its own profound regulatory failures, was ordered to pay substantial damages and established the ONIAM compensation fund, which allocated an initial €6.5 million package for victims, later expanding to cover hundreds of individual civil claims.

Sanofi has consistently maintained a highly defensive legal posture throughout these proceedings. The company vehemently argues that it was "proactive and transparent," asserting that it repeatedly requested product information updates from the French health authority in 2003, which were allegedly refused or delayed by the state until 2006. Sanofi insists that pharmaceutical companies cannot unilaterally alter patient leaflets without explicit regulatory approval, utilizing this bureaucratic bottleneck as its primary defense against the manslaughter and fraud charges.

3.3 Great Britain: Legal Aid Collapse and Regulatory Capture Allegations

In the United Kingdom, where the drug has been licensed since 1973 as Epilim, the pursuit of corporate accountability has been repeatedly stifled by the mechanics of the legal funding system and allegations of regulatory capture. A massive group litigation effort against Sanofi, comprising over 80 severely affected families, spectacularly collapsed in 2010 when the Legal

Services Commission abruptly withdrew £3 million in public funding just days before the High Court trial was scheduled to commence. This devastating withdrawal left thousands of British victims without a viable avenue for corporate redress, effectively shielding Sanofi from British tort liability and shifting the financial burden entirely onto the state welfare system and the victims themselves.

Sanofi's post-restriction marketing practices have also drawn severe regulatory scrutiny in the UK, fueling accusations of corporate arrogance. In 2021, a concerned healthcare professional filed a formal complaint with the Prescription Medicines Code of Practice Authority (PMCPA) and the MHRA regarding Sanofi's promotional activities on Twitter. The corporate tweet promoted Epilim and linked to a website intended for healthcare professionals that completely failed to adequately balance the therapeutic claims with the profound teratogenic risks, particularly regarding women of childbearing potential. The MHRA intervened, forcing Sanofi to completely withdraw the promotional website. This incident highlighted ongoing concerns regarding the pharmaceutical industry's aggressive marketing tactics and the potential for regulatory capture, wherein safety agencies are perceived as being historically too lenient on multinational drug manufacturers that wield massive economic influence.

Furthermore, the UK's Independent Medicines and Medical Devices Safety Review (the Cumberlege Review), published in 2020 under the title *First Do No Harm*, issued a scathing indictment of the entire healthcare system's failure to listen to women. The review explicitly called for the creation of an independent redress agency and a "costs of care" scheme funded jointly by the state and the manufacturer. However, the UK government initially rejected the core compensation recommendations, leading to years of agonizing, continued campaigning by affected mothers.

4. The Republic of Ireland Context: High Court Settlements and the O'Flaherty Inquiry

In the Republic of Ireland, sodium valproate was licensed in 1975, and its use was expanded to cover bipolar disorder in 2008. In the absence of a dedicated corporate or state compensation fund comparable to the French ONIAM scheme, legal action in Ireland has primarily targeted the Health Service Executive (HSE) and individual prescribing neurologists for medical negligence and breach of the duty to warn.

4.1 Landmark High Court Settlements

Despite the immense difficulties of pursuing individual medical negligence claims, significant breakthroughs have occurred in the Irish High Court. In March 2023, the High Court approved a landmark €15 million settlement for a 13-year-old boy, Jack Clarke, who developed severe autism and FVSD after his mother was prescribed Epilim during pregnancy. A secondary settlement of €2.65 million was approved for his younger brother, with a formal admission of liability by the defendant in the latter case.

Similarly, in another highly publicized case, Joshua Daly, a 12-year-old boy diagnosed with FVSD and autism spectrum disorder, secured a €3.75 million interim settlement against the HSE. His mother successfully argued that had she been adequately warned of the known teratogenic risks by her consulting neurologists at Cork University Hospital, she would have immediately opted for alternative therapeutic treatments. These multimillion-euro settlements underscore the sheer magnitude of the clinical negligence involved. However, because they are

achieved through adversarial tort litigation, they require immense financial resources, years of psychological endurance, and strict adherence to the statute of limitations, leaving the vast majority of affected families disenfranchised.

4.2 The O'Flaherty Inquiry: Timeline and March 2026 Status

The systemic failure to protect Irish women from sodium valproate led to intense, decade-long lobbying by advocacy groups, principally OACS Ireland (Organisation for Anti-Convulsant Syndromes) and Epilepsy Ireland. Following years of political pressure, the Irish Government committed to an inquiry in November 2020.

Despite the 2020 commitment, the establishment of the inquiry was marred by agonizing administrative delays. The Terms of Reference were not agreed upon until July 2023. The appointment of an independent Chair, Bríd O'Flaherty BL—a highly respected barrister with extensive experience in medical negligence and dispute resolution—did not occur until June 2024. The Department of Health frequently cited complex legal requirements, specifically the need to draft statutory instruments under the Data Protection Act 2018, to ensure the inquiry had a robust legal basis for processing highly sensitive medical data. Advocacy groups heavily criticized these justifications, viewing them as bureaucratic stalling that cruelly protracted the suffering of affected families.

The non-statutory inquiry formally commenced its operations on July 22, 2025. It was meticulously structured into three distinct operational phases:

1. **The Review Strand (Phase One):** Establishing a comprehensive, forensic timeline of the licensing, regulation, prescribing practices, and safety information dissemination of sodium valproate in the Irish State from 1975 to the present day.
2. **The Oral Hearings (Phase Two):** Collecting intimate testimonies and oral statements from individuals diagnosed with FVSD, their mothers, and families. This phase was scheduled to commence in September 2025, offering a private, highly supported forum for victims to formally document their trauma.
3. **The Recommendations Strand (Phase Three):** Assessing the health system's current capacity to manage anti-seizure medication safety and evaluating the absolute adequacy of existing supports (financial, medical, and educational) for FVSD victims, culminating in a final, actionable report to the Minister for Health.

As of early 2026, the inquiry is operating deep within its investigative and testimonial phases. In December 2025, OACS Ireland, supported by specialist medical negligence solicitors, hand-delivered their extensive, multi-volume submission—containing decades of harrowing patient testimonies and expert medical analysis—directly to the inquiry offices on Mount Street in Dublin. This was followed in February 2026 by a formal submission from Epilepsy Ireland. On March 13, 2026, the Minister for Health, Jennifer Carroll MacNeill, addressed Dáil Éireann to provide a formal, highly anticipated update on the inquiry's progress. The Minister confirmed that the nationwide public awareness campaign had concluded successfully in late 2025, and that the critical collection of oral statements from victims (Phase Two) was actively and sensitively proceeding. She explicitly noted that the Chair had established vital participant supports, including psychological counseling, legal guidance, and modest ex-gratia payments to facilitate victim engagement and mitigate the trauma of testifying. While the inquiry is definitively progressing, the victims and advocacy groups remain acutely, painfully aware that its non-statutory nature severely limits its ultimate legal and punitive authority.

5. Institutional Deflection: The Shadow of the Catholic Church Inquiries

A critical, sociological element in understanding the profound difficulty of obtaining comprehensive legal reparations for valproate victims in Ireland is the State's deeply ingrained jurisprudential reliance on the "non-statutory inquiry." To fully grasp this phenomenon, one must examine the historical precedent established by the Irish government during its investigations into the systemic institutional abuses perpetrated by the Roman Catholic Church.

5.1 The Blueprint of State Deflection

For the vast majority of the 20th century, the Irish State delegated the provision of health, education, and social care almost entirely to the Catholic Church and its affiliated religious orders. When the horrific, systemic physical, psychological, and sexual abuses within institutions such as the Magdalene Laundries, Industrial Schools, and various dioceses (as documented in the Ferns, Ryan, Murphy, and Cloyne reports) were finally exposed to the public, the State faced unprecedented legal, moral, and catastrophic financial exposure.

To meticulously manage this existential crisis, the Irish Government frequently bypassed full statutory tribunals—which possess the draconian powers of the Tribunals of Inquiry (Evidence) Act 1921—in favor of establishing commissions of investigation and non-statutory inquiries. While non-statutory inquiries (such as the McAleese Committee regarding the Magdalene Laundries) are often praised for being faster, more survivor-focused, and less aggressively adversarial, they suffer from fatal, deliberate legal limitations: they lack the absolute power to subpoena hostile witnesses under oath, they cannot compel the discovery of hidden corporate or institutional documents, and crucially, their findings cannot directly attribute civil or criminal liability to specific actors.

Scholars of transitional justice and legal sociology note that this specific investigatory approach allowed the Catholic Church—and by extension, the complicit State—to offer carefully crafted "apologies" and establish limited, tightly controlled financial redress schemes without exposing themselves to the catastrophic, limitless financial ruin of open-court tort litigation. It effectively shielded powerful institutions by framing systemic, willful negligence as a tragic historical oversight, rather than actionable corporate or state criminality.

5.2 Application to the Sodium Valproate Scandal

The sodium valproate crisis in Ireland is currently being managed using this exact historical blueprint. OACS Ireland and Epilepsy Ireland campaigned fiercely and relentlessly for a *statutory* inquiry. They sought a framework with the legal teeth to force the HSE, the Health Products Regulatory Authority (HPRA), and the executives of Sanofi to produce internal communications, safety data, and prescribing mandates under absolute legal compulsion. However, the Irish Cabinet flatly refused this demand, offering only a non-statutory inquiry as a final, non-negotiable ultimatum.

As Karen Keely, Chairperson of OACS Ireland, starkly stated upon the Cabinet's approval, the affected families "reluctantly agreed to proceed with a non-statutory inquiry" solely because they were "left in no doubt during our prolonged negotiations that the terms of reference agreed by Cabinet today were the only terms on offer". The State's unwavering insistence on this lesser

framework acts as a formidable, structural barrier to sweeping legal reparations. Because the O'Flaherty Inquiry is legally prohibited from making direct findings of civil or criminal liability, any mother whose child was catastrophically injured by valproate—or any patient misdiagnosed and harmed by the drug's severe side effects—must still bear the immense, crushing burden of proving medical negligence in the High Court. They are forced to fight the State Claims Agency and the HSE's aggressive defense attorneys one individual case at a time.

This institutional dynamic clearly explains why securing holistic legal reparations in Ireland remains incredibly difficult. The State utilizes the non-statutory inquiry to fulfill the performative, public-facing aspects of "truth-telling," "acknowledgment," and "listening to victims," while simultaneously keeping the formidable legal barricades of the adversarial court system fully intact to protect the national exchequer from mass pharmaceutical payouts.

6. Strategic Cross-Jurisdictional Litigation: Dual British-Irish Citizenship

For an individual holding both an Irish and a British passport, who has been subjected to the profound medical negligence of clinical misdiagnosis (e.g., being prescribed valproate for an unconfirmed neurological condition) and subsequent off-label usage (e.g., maintaining the prescription for intermittent migraines), the legal landscape is highly complex but offers distinct, strategic cross-jurisdictional options.

6.1 Establishing the Core Breach of Duty

In both the United Kingdom and the Republic of Ireland, prevailing in a medical negligence tort relies absolutely on demonstrating a fundamental breach of the duty of care that directly, causally resulted in harm. In the specific scenario of migraine misdiagnosis and valproate prescription, the plaintiff's legal counsel would aggressively target two primary clinical failures:

1. **Diagnostic Negligence and Erroneous Prescription:** Prescribing a highly toxic, known teratogenic antiepileptic drug for a condition the patient did not possess, thereby subjecting them to unwarranted hepatic, metabolic, and neurological risks.
2. **The Failure of Informed Consent and Protocol Adherence:** Continuing the toxic prescription for intermittent migraines without explicitly informing the patient of the severe teratogenic risks (FVSD), and categorically failing to implement the mandatory Pregnancy Prevention Programme (PPP).

Under the seminal legal precedent set by *Montgomery v Lanarkshire Health Board* (a ruling which strongly dictates modern medical law in both the UK and Ireland), physicians have an absolute legal obligation to inform patients of any "material risks" associated with a proposed treatment. Given that sodium valproate carries an undisputed 10% risk of major physical birth defects and a 40% risk of severe developmental disorders, the failure to obtain explicit, documented, and fully informed consent is a profound, indefensible breach of clinical duty.

6.2 The United Kingdom Legal Avenue: Redress Schemes vs. Direct Litigation

If the medical negligence (the misdiagnosis and failure to warn) occurred within the jurisdiction of the UK's National Health Service (NHS), the dual-citizen patient possesses two potential,

parallel pathways to justice:

- **The Hughes Report Redress Scheme:** Published in February 2024 by the Patient Safety Commissioner, Dr. Henrietta Hughes, this landmark report demands the UK government establish a comprehensive, two-stage financial redress scheme for valproate victims. The proposal advocates for an expedited, standardized interim payment of £100,000 to those harmed, followed by tailored, individualized financial and non-financial support packages. While the UK government has formally acknowledged the report's grim findings, the redress scheme is not yet fully operationalized by statute as of early 2026, leaving victims in a prolonged state of administrative and financial limbo.
- **Clinical Negligence Litigation:** A direct, adversarial lawsuit against the specific NHS Trust or the prescribing clinician. Claims involving historical pharmaceutical harm must carefully navigate the rigid confines of the Limitation Act 1980. However, UK courts frequently exercise judicial discretion regarding the "date of knowledge" in complex medical cases, recognizing that the full neurodevelopmental impact of FVSD on a child often does not become apparent until several years after birth, thus delaying the start of the limitation clock. Furthermore, under the UK Equality Act 2010 (and the Disability Discrimination Act 1995 in Northern Ireland), patients suffering from severe, misdiagnosed migraines are legally protected from discrimination, adding another layer of legal leverage regarding how their underlying condition was initially mismanaged.

6.3 The Irish Legal Avenue: High Court Tort Litigation

If the clinical negligence occurred within the Irish healthcare system, the UK's Hughes redress scheme is entirely inapplicable. The patient must pursue a complex medical negligence tort directly through the Irish High Court.

While the Irish system does not currently offer a dedicated, no-fault compensation scheme or a state-backed valproate fund, the legal precedent for massive, multi-million euro payouts has been firmly established by the €15 million Clarke settlement and the €3.75 million Daly settlement. Elite law firms in Dublin specializing in pharmaceutical negligence base these high-value claims on the catastrophic failure of Irish neurologists, obstetricians, and general practitioners to strictly adhere to the HPRA and EMA safety warnings issued sequentially between 2014 and 2018.

A dual citizen residing in Ireland, or whose misdiagnosis occurred under the auspices of the HSE, would file a plenary summons detailing both the profound psychological and physical injuries sustained by the child (FVSD), and the direct trauma inflicted upon the mother from the unnecessary, prolonged valproate exposure. Furthermore, if the mother developed secondary, permanent adverse effects from the unwarranted use of valproate for migraines—such as polycystic ovary syndrome, lasting liver toxicity, or chronic, debilitating withdrawal symptoms upon cessation—these constitute distinct, quantifiable damages within the broader personal injury claim, significantly maximizing the potential for comprehensive legal reparations.

7. Synthesis and Concluding Analysis

The global sodium valproate scandal represents a colossal, systemic failure of the international pharmacovigilance ecosystem, medical ethics, and corporate responsibility. The drug, while undeniably a potent intervention for severe, refractory epilepsy, possesses a toxicity and teratogenic profile so profound that its indiscriminate, off-label prescription to women of

childbearing potential—particularly for non-seizure conditions such as intermittent migraines—constitutes a massive breach of clinical duty.

The historical corporate actions of Sanofi—characterized by a highly reactive rather than proactive approach to patient safety, aggressive promotional tactics, and a calculated reliance on bureaucratic obfuscation regarding the updating of patient safety leaflets—have rightfully culminated in historic class-action verdicts and unprecedented criminal indictments for manslaughter in France. However, in common-law jurisdictions like the United Kingdom and the Republic of Ireland, the legal and political architecture has frequently insulated the manufacturer, transferring the crushing financial and moral burden of lifelong care to state healthcare providers and, ultimately, to the devastated families themselves.

In Ireland, the ongoing O'Flaherty Inquiry, which officially commenced in July 2025 and continues to gather harrowing oral evidence into the spring of 2026, is a vital, necessary step toward historical clarity and patient vindication. Yet, by deliberately forcing the adoption of a non-statutory framework, the Irish State has intentionally resurrected the precise defensive, liability-shielding methodologies it previously utilized to manage the fallout from the Catholic Church institutional abuse scandals. This structural decision ensures that while the historical truth of the valproate scandal may finally be documented, the path to actual legal reparations remains fiercely adversarial, requiring victims to endure the grueling, financially perilous gauntlet of High Court tort litigation.

For individuals subjected to the dual trauma of clinical misdiagnosis and subsequent off-label prescription, the breach of the duty of care is absolute and legally actionable across both British and Irish jurisdictions. The abject failure of the medical establishment to secure informed consent regarding a globally recognized teratogen demands rigorous legal redress. As the international medical and legal communities await the final recommendations of the Irish inquiry and the formal implementation of the UK's Hughes Report redress scheme, the pursuit of holistic justice for valproate survivors remains one of the most defining, urgent legal and ethical battles of the current decade.

Works cited

1. FAQs | Sodium Valproate Inquiry Ireland, <https://sodiumvalproateinquiry.ie/faq/> 2. Sodium Valproate Timeline – Key Events July 2020, <https://www.immdsreview.org.uk/downloads/Annexes/Annex-C-Valproate-timeline.pdf> 3. Sodium Valproate (Epilim) Claims: Understanding the Impact of Epilepsy Drug Use in Pregnancy - Sherwin O'Riordan, <https://sor-solicitors.ie/news-insights/sodium-valproate-epilim-claims/> 4. France's Sanofi charged with manslaughter in birth defects case - Al Jazeera, <https://www.aljazeera.com/economy/2020/8/3/frances-sanofi-charged-with-manslaughter-in-birth-defects-case> 5. Regarding the ongoing Depakine® (sodium valproate) product litigation in France, Sanofi is issuing the following statement, con, https://www.sanofi.com/assets/dotcom/pages/docs/our-company/social-impact/responsible-business-values/information-on-sodium-valproate/20220830_statement-on-depakine.pdf 6. Sodium Valproate – the future of claims and compensation | Medical Negligence and Personal Injury Blog | Kingsley Napley, <https://www.kingsleynapley.co.uk/insights/blogs/medical-negligence-and-personal-injury-blog/sodium-valproate-the-future-of-claims-and-compensation> 7. Sodium Valproate (Epilim) Inquiry - Healy Law, <https://www.healylaw.ie/sodium-valproate-epilim-inquiry> 8. Information on Post Authorization Safety Study (April 15, 2024) - Sanofi, https://www.sanofi.com/assets/dotcom/pages/docs/our-company/social-impact/responsible-business-values/information-on-sodium-valproate/20220830_statement-on-depakine.pdf

ness-values/information-on-sodium-valproate/information-on-Post-Authorization-Safety-Study-April-15-2024.pdf 9. Valproate (Sodium Valproate, Valproic Acid): Potential Risks to Offspring Following Paternal Exposure, and Male Infertility - NPRA, <https://www.npra.gov.my/index.php/en/component/content/article/465-english/safety-alerts-main/safety-alerts-2025/1527743-valproate-sodium-valproate-valproic-acid-potential-risks-to-offspring-following-paternal-exposure-and-male-infertility.html?Itemid=1391> 10. AUTH/3166/2/19 - Complainant v Sanofi, <https://www.pmcpa.org.uk/cases/completed-cases/auth3166219-complainant-v-sanofi/> 11. Solicitor representing Sodium Valproate patient groups comments on Sanofi charged with manslaughter - Leigh Day, <https://www.leighday.co.uk/news/press-releases/2020-news/solicitor-representing-sodium-valproate-patient-groups-comments-on-sanofi-charged-with-manslaughter/> 12. 8:50 am - UW-La Crosse, https://www.uwlax.edu/globalassets/offices-services/urc/presenting/2022_recs_abstract_book-4thannual.pdf 13. S.I. No. 352/2025 - Data Protection Act 2018 (Section 60(6)) (Inquiry into the Licensing and Use of Sodium Valproate in Women of Child-Bearing Potential in the State) Regulations 2025 - Irish Statute Book, <https://www.irishstatutebook.ie/eli/2025/si/352/made/en/print> 14. Inquiries | Transforming Justice Responses to Non-Recent Institutional Abuses | Oxford Academic, <https://academic.oup.com/book/59821/chapter/511215546> 15. Case Number: 202009-130982 | Department of Financial Services - NY DFS, <https://www.dfs.ny.gov/public-appeals/case-number-202009-130982> 16. New FDA Guidelines for Valproate Use in Pregnancy | InfantRisk Center, <https://www.infantrisk.com/new-fda-guidelines-valproate-use-pregnancy> 17. Neurology And Neurosurgery Negligence Claims Ireland: Complete Guide 2026, <https://www.personalinjurysolicitorsdublin.info/medical-negligence/neurology-neurosurgery/> 18. Sodium Valproate in the UK: Claims, Compensation, and Recent Court Cases Explained, <https://national-claims.co.uk/sodium-valproate-in-the-uk-claims-compensation-and-recent-court-cases-explained/> 19. Effect of Sodium Valproate Treatment on the Cardiac Index in New Cases with Status Epilepticus - The Open Neurology Journal, <https://openneurologyjournal.com/VOLUME/15/PAGE/59/FULLTEXT/> 20. Valproate - Wikipedia, <https://en.wikipedia.org/wiki/Valproate> 21. New valproate regulations, informed choice and seizure risk - PMC, <https://pmc.ncbi.nlm.nih.gov/articles/PMC11319429/> 22. Sodium Valproate (Epilim) Inquiry Update - Tormeys Solicitors, <https://tormeys.ie/sodium-valproate-epilim-inquiry-update/> 23. Valproate HCP Guide.indd - Sanofi, <https://www.sanofi.com/assets/countries/india/docs/science-and-innovation/for-healthcare-professionals/product-information/pharmaceuticals/valproate-hcp-guide.pdf> 24. Sanofi faces formal investigation over epilepsy drug Depakine - Pharmaceutical Technology, <https://www.pharmaceutical-technology.com/news/sanofi-depakine-formal-investigation/> 25. Sodium Valproate Claims - Osbornes Law, <https://osborneslaw.com/blog/sodium-valproate-claims/> 26. Epilepsy Society welcomes redress scheme for valproate victims but says research is also vital, <https://epilepsysociety.org.uk/news/clone-epilepsy-society-welcomes-redress-scheme-valproate-victims-says-research-also-vital> 27. Minister for Health provides update on Sodium Valproate inquiry - Epilepsy Ireland, <https://www.epilepsy.ie/content/minister-health-provides-update-sodium-valproate-inquiry> 28. Sodium Valproate Inquiry Officially Begins - Epilepsy Ireland,

<https://www.epilepsy.ie/content/sodium-valproate-inquiry-officially-begins> 29. Sanofi reports issues around safety study into valproate - Epilepsy Society, <https://epilepsysociety.org.uk/news/valproate-safety-study-concerns> 30. Sodium valproate | Epilepsy Society, <https://epilepsysociety.org.uk/about-epilepsy/sodium-valproate> 31. Valproate and reproductive risks in women and men with epilepsy: Dr. Tony Marson, <https://www.ilae.org/journals/epigraph/epigraph-vol-27-issue-3-summer-2025/valproate-and-reproductive-risks-in-women-and-men-with-epilepsy-dr-tony-marson> 32. Treatment of migraine attacks and preventive treatment of migraine - International Headache Society, https://ihs-headache.org/wp-content/uploads/2023/06/DMKG_Treatment-of-migraine-attacks-and-preventive-treatment-of-migraine-2022.pdf 33. Annex F: Sodium valproate Supporting Information, <https://www.immdsreview.org.uk/downloads/Annexes/Annex-F-Sodium-valproate-supporting-information.pdf> 34. Sodium valproate - NHS Resolution, <https://resolution.nhs.uk/sodium-valproate/> 35. New measures to avoid valproate exposure in pregnancy endorsed - EMA, <https://www.ema.europa.eu/en/news/new-measures-avoid-valproate-exposure-pregnancy-endorsed> 36. The mothers fighting a scandal bigger than thalidomide: 'We were told the medication was safe' - The Guardian, <https://www.theguardian.com/society/2024/feb/22/the-mothers-fighting-a-scandal-bigger-than-thalidomide-we-were-told-the-medication-was-safe> 37. Medication-Overuse Headache - StatPearls - NCBI Bookshelf - NIH, <https://www.ncbi.nlm.nih.gov/books/NBK538150/> 38. 11th European Headache Federation Congress jointly with 31st Congress of the Italian Society for the Study of Headaches: Rome, Italy. 01-03 December 2017 - PMC, <https://pmc.ncbi.nlm.nih.gov/articles/PMC5709272/> 39. French court rules against Sanofi in landmark Depakine case - Apesac, <https://apesac.org/presse/dans-la-presse-2022/french-court-rules-against-sanofi-in-landmark-depakine-case/> 40. Historic judgment rendered by the Paris Court in a French class action | BCLP, <https://www.bclplaw.com/en-US/events-insights-news/historic-judgment-rendered-by-the-paris-court-in-a-french-class-action.html> 41. Trial against epilepsy drug manufacturer collapses after legal aid is withdrawn - The BMJ, <https://www.bmj.com/content/341/bmj.c6384> 42. Promotion of Epilim (sodium valproate) by Sanofi - GOV.UK, <https://www.gov.uk/government/publications/advertising-investigations-may-2021/promotion-of-epilim-sodium-valproate-by-sanofi> 43. Sanofi pulls Epilim website after U.K. regulator points out consumer complaint, <https://www.fiercepharma.com/marketing/sanofi-pulls-epilim-website-after-u-k-regulator-points-out-consumer-complaint> 44. Valproate: 'Who knew what and when?' - Medical Independent, <https://www.medicalindependent.ie/in-the-news/news-features/valproate-who-knew-what-and-when/> 45. Court approves two settlements over epilepsy drug use - Michael Boylan LLP, <https://www.michaelboylan.com/court-approves-two-settlements-over-epilepsy-drug-use/> 46. MBLLF secure €3.75 million interim settlement for 12-year-old boy over use of epilepsy drug in mother's pregnancy - Michael Boylan LLP, <https://www.michaelboylan.com/mbllf-secure-e3-75-million-interim-settlement-for-12-year-old-boy-over-use-of-epilepsy-drug-in-mothers-pregnancy/> 47. Sodium Valproate (Epilim) Claims: Epilepsy drug use in pregnancy harms children, <https://aclsolicitors.ie/blog/epilepsy-drug-sodium-valproate-use-in-pregnancy-harms-children/> 48. Sodium Valproate (Epilim) Campaign - Epilepsy Ireland, <https://www.epilepsy.ie/content/sodium-valproate-epilim-campaign> 49. Chair of Inquiry into historical licensing and use of sodium valproate in women of child-bearing potential - Health

Manager,

<https://healthmanager.ie/2024/08/chair-of-inquiry-into-historical-licensing-and-use-of-sodium-valproate-in-women-of-child-bearing-potential/> 50. RTE News The real Life of a person impacted by Epilim (Sodium Valproate) - OACS Ireland, <https://oacsireland.com/rte-news-the-real-life-of-a-person-impacted-by-epilim-sodium-valproate/> 51. Epilim Sodium Valproate Birth Defects - Collier Law Solicitors, <https://collierlaw.ie/justice-for-valproate-epilim-survivors-inquiry-to-give-a-voice-to-those-who-had-no-choice/> 52. Departmental Inquiries – Thursday, 26 Feb 2026 – Parliamentary Questions (34th Dáil) – Houses of the Oireachtas, <https://www.oireachtas.ie/en/debates/question/2026-02-26/492/> 53. Sodium Valproate Inquiry Update (2025) - Epilepsy Ireland, <https://www.epilepsy.ie/content/sodium-valproate-inquiry-update-2025> 54. 2025 Sodium Valproate Inquiry in Ireland | SV Inquiry, <https://sodiumvalproateinquiry.ie/> 55. Submission Delivery to the Sodium Valproate Inquiry - OACS Ireland, <https://oacsireland.com/submission-delivery-to-the-sodium-valproate-inquiry/> 56. STATUTORY INSTRUMENTS. S.I. No. 351 of 2025 DATA PROTECTION ACT 2018 (SECTION 51(3)) (INQUIRY INTO THE LICENSING - Irish Statute Book, <https://www.irishstatutebook.ie/eli/2025/si/351/made/en/pdf> 57. Statement from OACS Ireland and Epilepsy Ireland on cabinet approval for non-statutory inquiry into historical licensing and prescribing of Sodium Valproate, <https://www.epilepsy.ie/content/statement-oacs-ireland-and-epilepsy-ireland-cabinet-approval-non-statutory-inquiry> 58. the ferns report - Bishop Accountability, https://www.bishop-accountability.org/reports/2005_10_Ferns/Complete_Ferns_Report_SO_Part_1.pdf 59. K.O. v. IRELAND - HUDOC - The Council of Europe, <https://hudoc.echr.coe.int/eng?i=001-192528> 60. The inquiry into institutional child abuse in Ireland, <https://dera.ioe.ac.uk/id/eprint/27492/1/4110.pdf> 61. Non-recent Institutional Abuses and Inquiries - Research Repository UCD, <https://researchrepository.ucd.ie/bitstreams/693d5c6b-d057-483e-a3e6-456271df561e/download> 62. Non-recent Institutional Abuses and Inquiries: Truth, Acknowledgement, Accountability and Procedural Justice - Public Policy.ie, <https://publicpolicy.ie/papers/non-recent-institutional-abuses-and-inquiries-truth-acknowledgement-accountability-and-procedural-justice/> 63. Assessing Transitional Justice for Historical Abuses of Church and State (Part II), <https://www.cambridge.org/core/books/transitional-justice-and-the-historical-abuses-of-church-and-state/8D55312791920AA646D76895708E70B9> 64. From shame to guilt: negotiating moral and legal responsibility within apologies for historical institutional abuse - Queen's University Belfast Research Portal, https://pure.qub.ac.uk/files/366794961/Journal_of_Law_and_Society_2022_MCALINDEN_From_shame_to_guilt_negotiating_moral_and_legal_responsibility_within.pdf 65. Sodium Valproate (Epilim) Claims - Healy Law, <https://www.healylaw.ie/sodium-valproate-epilim-claims> 66. Am I eligible to receive compensation if I have been affected by sodium valproate?, <https://www.slatergordon.co.uk/newsroom/am-i-eligible-to-receive-compensation-if-i-have-been-affected-by-sodium/> 67. I'm a clinical negligence solicitor; here is what you should know about sodium valproate - Slater and Gordon, <https://www.slatergordon.co.uk/newsroom/im-a-clinical-negligence-solicitor-here-is-what-you-should-know-about-sodium/> 68. The Hughes Report: Options for redress for those harmed by valproate and pelvic mesh - Patient Safety Commissioner, https://www.patientsafetycommissioner.org.uk/wp-content/uploads/2024/02/14.385_DHSC_OPS

SC-Redress-Report.pdf 69. Migraine, disability and legal rights,
<https://migrainetrust.org/live-with-migraine/migraine-disability-and-legal-rights/> 70. Valproate
Sodium Side Effects: Common, Severe, Long Term - Drugs.com,
<https://www.drugs.com/sfx/valproate-sodium-side-effects.html> 71. Side effects of sodium
valproate - NHS,
<https://www.nhs.uk/medicines/sodium-valproate/side-effects-of-sodium-valproate/>